

CALIMESA FIRE DEPARTMENT

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- xv. Controlling hemorrhage
- xvi. Bandaging wounds
- xvii. Splinting fractures
- xviii. Packing and handling patients
- xix. Axial spinal stabilization
- xx. Mast application & removal
- xxi. Needle cricothyroidotomy procedures
- xxii. Mouth to adjunct ventilation
- xxiii. Mouth to mask ventilation
- xxiv. Bag valve mask ventilation procedures
- xxv. Positive pressure ventilation
- xxvi. Decontamination of equipment or laundry
- xxvii. General Housekeeping
- xxviii. Transport of Bio-Hazardous waste
- xxix. Rescuing victims from hostile environments, including burning structures or vehicles, water contaminated atmospheres, or oxygen-deficient atmospheres
- xxx. Extrication of persons from vehicles, machinery, or collapsed excavations or structures
- xxxi. Recovery or removal of bodies from any situation cited above
- xxxii. Response to hazardous materials incidents, of either transportation-related or fixed-site, of potentially infectious substances
- xxxiii. The above list may be revised and updated as the tasks, procedures, and classifications change according to the Fire Department's present need

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B. The following job classifications are high risk to be involved in exposure to blood, body fluids, other potentially infectious material in the performance of their duties:

- i. Firefighter
- ii. Fire Captain
- iii. Intern Reserve Firefighter
- iv. Battalion Chief
- v. Deputy Fire Chief
- vi. Fire Chief
- vii. Administrative Assistant I
- viii. Administrative Assistant II
- ix. Senior Administrative Assistant

IV. RESPONSIBILITIES/DUTIES

A. The following are the three (3) major "Categories of Responsibility" that are central to the effective implementation of the Exposure Control Plan for Bloodborne Pathogens Policy.

- i. City Manager
- ii. Fire Department Safety Officer
- iii. Fire Department Employees

B. The following sections define the roles played by each of the above personnel in carrying out the Exposure Control Plan for Bloodborne Pathogens Policy.

- i. City Manager
 - a) Ultimate responsibility for the health and safety of the Fire Department members
 - b) Designates a Fire Department Safety Officer

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ii. Fire Department Safety Officer

- a) Oversee the development and management of the Fire Department Exposure Control Plan for Bloodborne Pathogens Policy
- b) Work with the City of Calimesa and Fire Department's appointed Physician to coordinate Fire Department's immunization programs as necessary for Fire Department employees' health and welfare. Maintain copies of such immunization records in either the Fire Department's Medical Confidential files and the Fire Department's appointed physician's office
- c) Maintain a confidential and non-confidential database of exposure and treatment given in conjunction with the Exposure Control Plan for Bloodborne Pathogens Policy
- d) Serve as the Department's "designated Officer" as required by the Ryan White Comprehensive AIDS Resources Act of 1990
- e) Evaluate possible employees' occupational exposure to potentially infectious material and coordinate communications between the Fire Department, area hospitals, and County Public Health
- f) Collect quality assurance data on the Exposure Control Plan for Bloodborne Pathogens Policy and present the data.
- g) Conduct spot inspections of on-scene and station operations to ensure compliance with the Exposure Control Plan for Bloodborne Pathogens Policy
- h) Collect and maintain a suitable reference library on the bloodborne pathogens standard and bloodborne pathogens safety and health information
- i) Be knowledgeable on current legal requirements concerning bloodborne pathogens and infectious diseases
- j) Act as the Fire Department's liaison during OSHA inspections
- k) Conduct audits to maintain an up-to-date Exposure Control Plan for Bloodborne Pathogens Policy

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- l) Work with the Fire Department to provide information and set up training for all employees at risk of exposure to bloodborne pathogens or infectious diseases
- m) Keep the City Manager abreast of significant problems or situations within the Fire Department associated with the Exposure Control Plan for Bloodborne Pathogens Policy

iii. Fire Department Employees

- a) Assume ultimate responsibility for their health and safety
- b) Aware of tasks that have a potential for occupational exposure to a communicable disease
- c) Attend Communicable and Bloodborne pathogens training sessions as required
- d) Know and follow the work practice controls outlined in the Exposure Control Plan for Bloodborne Pathogens Policy
- e) Report any suspected occupational exposure to a communicable disease to their immediate supervisor
- f) Report any diagnosis of communicable disease (occupational or non-occupational) to the Fire Department Safety Officer.

V. PLAN DEFINITIONS

BLOOD means human blood and its components

BLOODBORNE PATHOGENS are microorganisms transmitted through the bloodstream or body fluids

CONTAMINATED is the presence or the reasonably anticipated presence of blood or other potentially infectious materials

ENGINEERING CONTROLS are the means to isolate or remove bloodborne pathogens from the workplace. It is a physical control to reduce the chance of exposure

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EPA APPROVED GERMICIDAL SOLUTIONS are any product or substance in liquid, aerosol, or solid form that destroys or kills mycobacterium tubercle bacilli and immune deficiency virus

DECONTAMINATION uses physical or chemical to remove, inactivate, or destroy pathogens on a surface or item to the point where they are no longer capable of transmitting infectious particles, and the surface or item is rendered safe for handling, use, or disposal

SOURCE INDIVIDUAL is any individual, living or dead, whose blood or other body parts may be a source of occupational exposure or potentially infectious materials to the employee.

UNIVERSAL PRECAUTIONS is an approach for infection control applied to all patients, whereas blood and body fluids are considered infectious. The approach is designed to minimize the transmission of communicable disease when in direct contact with blood and body fluids

HAND WASHING means using an adequate supply of running water, soap, and a single-use towel or hot air drying machine to decontaminate hands

OCCUPATIONAL EXPOSURE is a situation in which body fluids, such as blood, saliva, semen, vaginal secretions, cerebrospinal fluids, synovial fluid, pericardial fluid, amniotic fluid, urine, or feces, of a patient have entered the body of an employee through a body opening such as a mouth or a nose, a mucous membrane and eyes, or a break in the skin from cuts, abrasions, a contaminated needle stick or scalpel

PERSONAL PROTECTIVE EQUIPMENT is the specialized clothing or equipment worn by an employee for protection against an exposure hazard.

REGULATED WASTE is the materials that are potentially infectious which could release blood, body fluids, or components thereof. The materials are in a liquid or semi-liquid state if compressed.

WORK PRACTICE is a control plan, which is designed to eliminate or minimize employee exposure.

EXPOSURE CONTROL PLAN is a plan of action that is designed to eliminate or minimize employee exposure.

H.I.V. is the Human Immunodeficiency Virus causing AIDS

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VI. AVAILABILITY OF THE EXPOSURE CONTROL PLAN TO PERSONNEL

- A. The Fire Department's Exposure Control Plan for Bloodborne Pathogens policy is available to all personnel to review at any time.
- B. Fire Department personnel are informed of the availability to review the Exposure Control Plan for Bloodborne Pathogens policy during their education/training sessions. The Exposure Control Plan for Bloodborne Pathogens policy is available for review in the following locations:
 - i. Fire Chief's Administration Office.
 - ii. City's Human Resource Office.
 - iii. Fire Station's Administration Office/Work Location (near mandatory posting area)

VII. REVIEW AND UPDATE OF THE PLAN

- A. The Fire Department's Administration recognizes the importance of keeping the Exposure Control Plan for Bloodborne Pathogens policy up-to-date.
- B. The plan will be reviewed and updated under the following circumstances:
 - i. Annually
 - ii. When a new task or procedure regarding exposure control plan for bloodborne pathogens are implemented

VIII. HEALTH MAINTENANCE

- A. No employee will be assigned to emergency response duties until the employee is certified fit for assignment by the Fire Department Physician or his/her designee
- B. Hepatitis B Vaccinations
 - i. Hepatitis B vaccinations are offered to employees
 - ii. An explanation of the vaccination risks and benefits will be explained to all employees and informed consent before immunization

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- iii. An employee may request serologic testing before Hepatitis B immunization to determine if previous immunity exists
- iv. Employees may refuse immunizations or submit proof of previous immunizations.
- v. Employees who refuse immunization will be counseled on the occupational risks of communicable disease and required to sign a refusal of immunization form
- vi. Employees may later receive immunization upon request

C. Current Immunizations

- i. All employees should have current immunizations or update of immunizations on file with Fire Department medical records
 - a) Measles.
 - b) Mumps.
 - c) Rubella.
 - d) Poliomyelitis (polio).
 - e) Tetanus.
 - f) Diphtheria.
- ii. An explanation of the vaccination risks and benefits will be explained to all employees and informed consent before immunization

D. ROUTINE HEALTH MAINTENANCE

- i. All employees will participate in a yearly screening for tuberculosis exposure.
- ii. All employees will participate in annual health evaluations

E. RELEASE FOR WORK

- i. Any employee returning to work following reportable communicable disease exposure (occupational or non-occupational) will need clearance by the Department Physician or designee before resuming emergency response duties

F. MEDICAL RECORDS

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- i. The Fire Department Safety Officer and the Fire Department Physician will maintain records under the FED-OSHA, C.F.R. 29 Part 1910.1030 and CAL-OSHA, Section 5193. Employees participating in the Infection Control Program documentation will including the following:
 - a) Employee's name and social security number
 - b) Immunization records
 - c) Circumstances of exposure to communicable diseases
 - d) Post-exposure medical evaluation, treatment, and follow-up
- ii. Infection Control records will become a part of the member's personnel file. The records will be maintained for a duration of employment plus thirty (30) years.
- iii. Medical records are strictly confidential.
 - a) Medical records will be maintained in the Department Physician's office and will not be filed with personnel records.
 - b) Medical records will not be released without the written consent of the employee. There are no exceptions to this policy for releasing medical information, including Fire Department Administration and Insurance companies.
- iv. Employees may examine their medical records
- v. Abstracts of medical records without personal identifiers may be used for:
 - a) Quality assurance
 - b) Compliance monitoring
 - c) Program evaluation purposes

IX. INFECTIOUS CONTROL TRAINING

- A. Employees providing emergency medical services will be required to complete infectious control training:
 - i. The initial time of assignment where occupational exposure may occur
 - ii. Refresher training at least annually

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- B. All Infection Control Training materials will be appropriate in content and vocabulary at the training's educational level, literacy, and language.
- C. Training will be available for employees that complies with:
 - i. NFPA Standard 1581
 - ii. FEDERAL OSHA Regulation 29 C.F.R. Part 1910.1030
 - iii. CAL-OSHA Regulation- California Code of Regulation Title 8: General Industry Safety Orders, Chapter 4, Section 5193.
- D. An explanation of the contents of the above regulations will be given and defined, including items listed below:
 - i. A general explanation of the epidemiology and symptoms of bloodborne diseases
 - ii. An explanation of the modes of transmission of bloodborne pathogens
 - iii. An explanation of the Fire Department's exposure control plan and how the employee can obtain a copy
 - iv. Information on types, proper use, location, removal, handling, decontamination, and disposal of personal protective equipment
 - v. Information on the appropriate actions to take and persons to contact in case of an exposure involving blood or potentially infectious materials
 - vi. An explanation of the procedures to follow if an exposure incident occurs
 - vii. An explanation of signage, labels, and color-coding required for biohazard materials. Information on proper storage and disposal of biohazard materials
 - viii. Opportunity for interactive questions and answers
 - ix. An explanation of the appropriate methods for recognizing tasks and other activities that may involve exposure to blood and other potentially infectious materials

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- x. An explanation of the use and limitations of methods that will prevent or reduce exposure, including appropriate engineering controls, work practices, and personal protective equipment
 - xi. An explanation of the basis for the selection of personal protective equipment
 - xii. Information on Hepatitis B vaccine, including information on efficiency, safety, method of administration, the benefits of being vaccinated, and will be provided free of charge by employer
- E. Infection control Trainers shall be knowledgeable in all of the program elements listed above, particularly as they relate to all emergency services provided by the Department
- F. Written records of training sessions will be maintained for three (3) years after the date on which the training occurs
- G. Training records will include:
 - i. The dates of training sessions
 - ii. The contents or a summary of the training sessions
 - iii. The names and qualifications of persons conducting the training
 - iv. The names and job titles of all persons attending the training sessions

X. DECONTAMINATION

A. Sterilization

- i. Destroys: All forms of microbial life including high numbers of bacterial spores
- ii. Methods: Steam under pressure (autoclave), ethylene oxide (gasclave), dry heat, or immersion in an E.P.A. approved chemical sterilant for a prolonged period of time (e.g., 6-10 hours) or according to manufacturer's instructions. The Department will use a germicidal solution according to the manufacturer's instructions
- iii. Use: For those instruments or devices that penetrate skin or contact normally sterile areas of the body (e.g., scalpels, needles, etc.) disposable invasive

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equipment eliminates the need to sterilize these types of items. However, arrangements can or should be made with the Department Exposure Control Officer to have the sterilization of reusable instruments accomplished

B. High-Level Disinfection

- i. Destroys: All forms of microbial life except high numbers of bacterial spores
- ii. Methods: Hot water pasteurization (176°F- 212°F) for 30 minutes, or exposure to E.P.A. registered sterilant as above for a short exposure time (e.g., 10-45 minutes) according to manufacturers' instructions
- iii. Use: For reusable instruments or devices-that come into contact with mucous membranes (e.g., laryngoscope blades, endotracheal tubes, etc.)

C. Intermediate-Level Disinfection

- i. Destroys: Mycobacterium, tuberculosis, vegetative bacteria, most viruses, and most fungi, but does not kill bacterial spores
- ii. Methods: E.P.A. registered hospital disinfectants, chemical germicides that have a label claim for tuberculocidal activity, commercially available hard surface germicides or solutions containing at least 1000-ppm free chlorine, or 1:10 dilution of common household chlorine bleach (approximately 1 cup bleach per gallon of water)
- iii. Use: For those surfaces that come into contact only with "intact skin" (e.g., stethoscopes, blood pressure cuffs, splints, backboards, etc.) and does not have visible material on it before the germicidal chemical is applied for disinfection

D. Low-Level Disinfection

- i. Destroys: Most bacteria, some viruses, some fungi, but not Mycobacterium tuberculosis or bacterial spores
- ii. Methods: E.P.A. registered hospital disinfectants (no label claim for tuberculocidal activity) 1:100 dilution of common household chlorine bleach (approximately 1/4 cup bleach per gallon of water)
- iii. Use: These agents are excellent cleaners and can be used for routine housekeeping or removal of soiling in the absence of visible fluid contamination.

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XI. STATION ENVIRONMENT

A. The Centers for Disease Control state that environmental forms of dried blood or contaminated needles and instruments may transmit disease. Housekeeping controls are necessary to ensure that employees' occupational exposure to blood or other infectious materials minimized

B. Environmental Disinfection

i. Environmental surfaces that have become soiled should be cleaned and disinfected using any cleaner or disinfectant agent intended for environmental use. Such surfaces include:

- a) Floors
- b) Woodwork
- c) Vehicle exteriors
- d) Vehicle interiors, including seats, and countertops, etc.

ii. To improve the effectiveness of any sterilization or disinfection process, containmanated equipment must first be thoroughly cleaned of all visible soil

iii. DO NOT USE CHLORINATED BLEACH ON TURNOUT CLOTHING, UNIFORMS, OR MAST SUITS.

C. Equipment Cleaning

i. Equipment cleaning and disinfecting shall not be conducted in the kitchen, living, sleeping, or personal hygiene areas

ii. Equipment cleaning and disinfecting shall be conducted in a designated decontamination area of each fire station

iii. Work surfaces shall be cleaned and decontaminated with 10% chlorinated bleach solution, disinfectant cleaner, or disinfectant agent after contact with blood or other infectious materials and after completion of equipment cleaning and disinfecting procedures

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- iv. All non-disposable equipment used on a patient will be cleaned after each use. Any equipment that had direct contact with the patient shall be cleaned with an E.P.A. approved germicidal solution
- v. The E.P.A. approved germicidal detergent will be used for cold soaking of the Laryngoscope blades. The rest of the equipment will be cleaned with hot soapy water and rinsed with hot water. Follow the cleaning with a 10% bleach solution, disinfectant cleaner, or disinfectant agent for disinfecting. Avoid immersing electrical equipment
- vi. Disposable items are one-time-use items. They shall be disposed of in the appropriate biohazard receptacle
- vii. Electrical and electronic equipment may be decontaminated by autoclave or gasclave
- viii. If contamination of a vehicle occurs, the contaminated areas are to be washed and hot soapy water. Rinse and dry thoroughly
- ix. Hand washing is MANDATORY before preparing food at the station and after removal of personal protective equipment

D. Uniforms and Laundry

- i. Washing of contaminated uniforms, specialized clothing, and laundry shall be washed at the fire station's wash area facility
- ii. Under no circumstances will contaminated work clothes be washed at home
 - a) Mandated by FED-OSHA Regulation 29 C.F.R. Part 1910.1030 Section (d)(3)(iv)(a)
 - b) Mandated by CAL-OSHA Regulation California Code of Regulation Title 8
 - c) General Industry Safety Orders, Chapter 4, Section 5193. (3)(A)(D).
- iii. Fire Station shall maintain a clean laundry area with a functional washer, dryer, and wash sink (as recommended per NFPA 1581)

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- iv. All employees shall maintain additional clean work uniforms in the station to allow of changing of contaminated uniforms or clothing upon return of incident

E. Kitchen and Restrooms/ Shower rooms

i. Kitchen

- a) Each station shall ensure that its refrigerators and freezers are in good working order and cleaned out periodically to avoid bacteria and other harmful contaminants. A routine cleaning schedule shall be practiced
- b) Food preparation areas, such as countertops and cutting boards, will be constructed of nonporous materials (Wooden cutting boards are no longer allowed)
- c) Foods shall be properly prepared and cooked. Food preparers shall wash their hands thoroughly before and after, handling and preparing food
- d) Food preparers shall use either liquid, or granulated soap during hand washing. **NO BAR SOAP IS ALLOWED**

ii. Restrooms/ Shower rooms

- a) Restrooms and shower rooms shall be cleaned daily, using appropriate E.P.A. approved germicidal and disinfectants
- b) Disposable hand drying materials shall be used in restrooms areas. **NO CLOTH TOWELS WILL BE USED IN DRYING OF HANDS**
- c) Restrooms and shower rooms shall have liquid or granulated soap dispensers for personnel usage. **NO BAR SOAPS ALLOWED**

F. Medical waste storage

- i. The Department shall designate an enclosure or accumulation area for the storage of medical waste containers and sharp containers
 - a) Contaminated sharps will consist of closed puncture-resistant containers (sharps boxes) with appropriate biohazard markings and color-coding
 - b) Waste containers will consist of leakproof material with appropriate biohazard markings and color codes.

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- ii. These areas shall be secured to prevent access to unauthorized persons and shall be marked with warning signs on or adjacent to the exterior of entry doors, gates, or lids
 - a) Warning signs shall be in English and Spanish, in addition to any other language determined appropriate by the infection control staff
 - b) Infectious waste storage areas will be marked with bio-hazard signs and maintained following all E.P.A. and local regulations, under Health & Safety Code, Division 20, Chapter 6.1, California Medical Waste Management Act, Section No. 25086; mandated by FED-OSHA Regulation 29 C.F.R., Part 1910.1030 Section (g)(1)(ii); mandated by C.A.L.- OSHA Regulation California Code of Regulation Title 8, General Industry Safety Orders, Chapter 4, Section 5193. (3)(A) LABELS(8)(B) SIGNS(1).
- iii. The storage area may be secured by the use of locks on entry doors, gates, or receptacle lids for each station pursuant to Health & Safety Code, Division 20, Chapter 6.1 and California Medical Waste Management Act Section No. 25086
- iv. All disposal of biohazard waste will be in accordance with E.P.A. and local regulations Fire Chief, designee, or an approved licensed contractor
 - a) Refer to State and County Management Acts

G. Decontamination Areas

- i. Decontamination areas shall have the following:
 - a) Proper lighting and adequate ventilation
 - b) Appropriate containers for storage of biohazard waste
 - c) Facilities for the safe storage, use, and disposal of cleaning and disinfecting solutions
 - d) Appropriate personal protection equipment for the use of disinfecting solutions
 - e) Adequate counter space constructed of nonporous materials

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- f) Sink made of nonporous materials, spray attachment optional, although highly recommended
- g) Safety data sheets for cleaning and disinfecting solutions. All personnel using these solutions will be familiar with the safety data sheets and the appropriate personal protective equipment
- h) Decontamination areas will be marked with bio-hazard signs, pursuant to Health & Safety Code, Division 20, Chapter 6.1, California Medical Waste Management Act, Section No. 25086; mandated by FED-OSHA Regulation 29 C.F.R., Part 1910.1030 Section (9)(1)(ii); mandated by CAL-OSHA Regulation California Code of Regulation Title 8, General Industry Safety Orders, Chapter 4, Section 5193.(3)(A)LABELS(8)(B)SIGNS(1).

XII. PROTECTIVE EQUIPMENT

- A. The Fire Department Safety Officer will develop personal protective equipment standards
- B. The Fire Department is responsible for the supply, repair, replacement, and safe disposal of Infection Control personal protection equipment
- C. The Fire Department Safety Officer will determine proper stock supply levels of personal protection equipment both for station inventory and for response vehicles inventory
 - i. The type and location of personal protection equipment will be standardized as much as possible on all response vehicles
- B. Available personal protection equipment
 - i. In addition to structural/ wildland firefighting and rescue personal protective equipment, the availability of personal protection equipment will include:
 - a) Disposable medical examination gloves
 - 1) Disposable gloves will be constructed of latex-free material with various sizes available
 - b) Face masks

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- c) Eye Protection
- d) Fluid-impervious gowns
- e) Sharps containers
 - 2) The sharps container will be a closeable puncture-resistant container with appropriate biohazard markings and color-coding and immediately accessible
- h) Leakproof biohazard bags
- i) Shoe covers

XIII. SCENE OPERATIONS

A. Precautions

- i. The patient's blood and body fluids are considered potentially infectious. As a result, universal precaution needs to be practice on all patient contacts
- ii. Employees are encouraged to use maximum rather than minimal personal protective equipment for each situation.
- iii. While complete control of the emergency scene is not possible, scene operations should limit splashing, spraying, or aerosolization of blood and body fluids
- iv. Minimize the number of employees required to complete tasks safely during on-scene operations
 - a) Employees not immediately needed will remain safe from operations, where communicable disease exposure is not possible or anticipated
- v. While at the scene of a medical aid incident, the following are prohibited:
 - a) Eating
 - b) Drinking
 - c) Smoking

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- d) Handling contact lenses
 - e) Applying cosmetics or lip balm
- vi. Employees will wash their hands following these activities:
 - a) After removing personal protective equipment
 - b) After each patient contact
 - c) After handling, potentially infectious materials
 - d) After cleaning or decontaminating equipment
- vii. While washing hands, pay particular attention to the fingernails hygiene
- viii. HAND WASHING SHALL PERFORM IN DESIGNATED BIOHAZARD AREAS, NOT IN THE KITCHEN AREA
- ix. Employees are discouraged from recapping, bending, or breaking syringe needles. Employees will not separate the syringe needle from the syringe

B. Disposable medical examination gloves

- i. Replace disposable medical examination gloves as soon as possible when soiled, torn, or punctured
- ii. Disposable medical examination gloves will not be reused, washed, or disinfected for reuse
- iii. Wear structural/ wildland firefighting gloves over disposable medical examination gloves in situations where sharp or rough edges are likely to be encountered

C. Facial protection

- i. Full Facial protection is afforded by using both a facemask and eye protection or by using a full-face shield

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- ii. Utilize particulate respirators when treating a patient with a suspected or known airborne transmissible disease

D. Clothing protection

- i. Fluid-impervious Gowns
- ii. Structural/ wildland Firefighting Gear
 - a) Structural/ wildland firefighting gear protects clothing from splashes and is preferable in fire, rescue, or vehicle extrication activities

E. Sharps container

- i. Dispose of all used needles and other sharps in approved sharps containers
- ii. Sharps containers will be easily accessible on-scene.

F. Shoe covers

- i. Protects footwear from potential contamination
- ii. Structural firefighting gear (waterproof) boots also may be used for barrier protection.

G. Contaminated items

- i. Contaminated items, such as gloves, gowns, airway adjuncts, and bandaging material, shall be collected, placed into a red biohazard bag, and disposed of appropriately
- ii. The Company Officer or scene supervisor shall ensure that nothing, such as contaminated bandage material, patient clothing if contaminated with blood, or other bodily fluids, needles, or any such item that may pose a threat to the public, shall not be left unattended at the emergency scene
- iii. Personal protective equipment, if contaminated, will be removed as soon as practicable

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XIV. POST-RESPONSE

- A. Upon conclusion of the incident and as soon as possible, all potentially contaminated reusable equipment will be removed, decontaminated, and placed back into service.

- B. Cleaning and storage
 - i. Infection contaminated equipment will be stored only in the decontamination area
 - ii. Infection biohazard waste generated during on-scene operations will be stored in the biohazard disposal area in appropriate leak proof containers
 - iii. Gloves will be worn for all contact with contaminated equipment or materials
 - iv. Other personal protective equipment will be used depending on splash or spill potential
 - v. Heavy-duty utility gloves may be used for cleaning, disinfection, or decontamination of equipment
 - vi. Eating, drinking, smoking, handling contact lenses, or applying cosmetics or lip balm is prohibited during cleaning or decontamination procedures
 - vii. Disinfection will be performed with an approved disinfectant or with a 1:100 solution of bleach in water. All disinfectants will be tuberculocidal and E.P.A. approved and registered
 - viii. Any damaged equipment will be cleaned and disinfected before being sent out for repair
 - ix. The manufacturer's guidelines will be used for the cleaning and decontamination of all equipment. Unless otherwise specified

- C. Infection contaminated structural/ wildland firefighting gear
 - i. Clean infection contaminated structural/ wildland firefighting gear according to manufacturer's recommendations

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- ii. Contaminated boots will be brush-scrubbed with a hot solution of soapy water, rinsed with clean water, and allowed to air dry
- D. Contaminated work clothes
 - i. Uniform shirts and pants will be removed and exchanged for clean clothes
 - ii. Wash infection contaminated work clothes according to manufacturer's recommendations
- E. Vehicles' interiors components should be scrubbed with an approved disinfectant when they have been in contact with contaminated clothing, turnout gear, or other equipment.

XV. EMPLOYEE OCCUPATIONAL COMMUNICABLE DISEASE EXPOSURE PROTOCOLS

- A. An employee occupational exposure to communicable disease consists of the following:
 - i. A break of intact skin by a potential infectious communicable disease material
 - ii. Blood, bodily fluids, or other potentially infectious communicable disease material onto eyes, mucous membranes, or non-intact skin.
- B. Any employee exposed to potentially infectious material will immediately wash the exposed area with soap and water or saline eyewash if the eyes are involved
- C. Any employee having an occupational communicable disease exposure will immediately report the exposure to his or her supervisor.
- D. The employee's supervisor will immediately the On-duty Officer and Fire Department Executive Management of the occupational communicable disease exposure and ensure a County of Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) and Calimesa F.D. Exposure Incident Report Form (Form 408.04) are filled out as soon as practical and forward to Executive Department Management completion of shift
 - i. The Calimesa F.D. Exposure Incident Report Form (Form 408.04) will include details of the task being performed, the means of transmission, the portal of entry, and the type of personal protective equipment use at the time.

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E. The Fire Department Executive Management will evaluate the County of Riverside Department of Public Health Communicable Disease Exposure Request Form and Calimesa F.D. Exposure Incident Report Form to determine if an occupational communicable disease exposure occurred

- i. If an occupational communicable disease exposure occurred, the Fire Department Executive Management would institute the following:
 - a) Designate a Ryan White Act Infections Disease Officer to conduct patient source tracing at the receiving medical facility, inform the receiving facility an occupational communicable disease exposure occurred with the source patient, and request a communicable disease determination, as provided under the Ryan White Act of 1990.
 - 1) The source patient has the right to refuse such testing under present regulations
 - b) Arrange medical evaluation by Workman's Comp physician in cooperation with the Fire Department Physician or designee no later than 48 hours post-exposure
 - c) If needed, refer the employee to an Employee Support and Health and Welfare program
 - d) Counsel the employee on exposure hazards
 - e) Complete the communicable disease exposure report, indicating the disposition of medical management
- ii. If an occupational communicable disease exposure did not occur, the Fire Department Executive Management would institute the following:
 - a) Counsel the employee on exposure hazards
 - b) If needed, refer the employee to an Employee Support and Health and Welfare program
 - c) Complete the communicable disease exposure report, indicating the disposition of medical management

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XVI. AREA HOSPITALS POINT OF CONTACT FOR REQUESTS A COMMUNICABLE DISEASE DETERMINATION

Desert Regional Medical Center

1150 N. Indian Canyon Dr. Palm Springs, CA 92262

Business Phone: (760) 323-6511

Emergency Department: (760) 323-6521

Request a communicable disease determination point of contact:

Contact On Duty Clinical Manager, explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to Pre Hospital Liaison Nurse

Kaiser Permanente Moreno Valley Medical Center

27300 Iris Avenue, Moreno Valley, CA 92555

Business Phone: (951) 243-0811

Emergency Department: (951) 251-6565

Request a communicable disease determination point of contact:

Contact either Emergency Department, Infection Control Officer, on-duty Charge Nurse, on-duty Clinical Manager, or Pre-hospital Liaison Nurse; explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to the the contact person

Riverside University Health System Medical Center

26520 Cactus Ave, Moreno Valley, CA 92555

Business Phone: (951) 486-4000

Emergency Department: (951) 486-5650

Request a communicable disease determination point of contact:

Contact either Emergency Department, Infection Control Officer, on-duty Charge Nurse, on-duty Clinical Manager, or Pre-hospital Liaison Nurse; explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to the the contact person

San Geronio Memorial Hospital

600 N Highland Springs Blvd, Banning, CA 92220

Business Phone: (951) 845-1121

Emergency Department: (951) 769-2121

Request a communicable disease determination point of contact:

Contact either Emergency Department, Infection Control Officer, on-duty Charge Nurse, on-duty Clinical Manager, or Pre-hospital Liaison Nurse; explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to the the contact person

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Loma Linda University Medical Center

11234 Anderson Loma Linda, CA. 92354

Business Phone: (909) 558-4000

Emergency Department: (909) 824-4444

Request a communicable disease determination point of contact:

Contact either Emergency Department, Infection Control Officer, on-duty Charge Nurse, on-duty Clinical Manager, or Pre-hospital Liaison Nurse; explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to the the contact person

Redlands Community Hospital

350 Terracina Drive, Redlands, CA 92373

Business Phone: (909) 335-5500

Emergency Department: (909) 335-5600

Request a communicable disease determination point of contact:

Contact either Emergency Department, Infection Control Officer, on-duty Charge Nurse, on-duty Clinical Manager, or Pre-hospital Liaison Nurse; explain the situation, be prepared to submit Riverside Department of Public Health Communicable Disease Exposure Request Form (Form DOH-CD-145) to the the contact person

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PURPOSE

This policy is intended to provide direction for the development of effective Divisional Traffic Control Programs. In accordance with Cal-OSHA requirements.

Adherence to this policy is intended to protect all workers and the public:

1. In work zones where Fire Department work is conducted in and amongst live lanes of vehicle traffic that may endanger motorists, workers, cyclists or pedestrian safety
2. In roadway allowances, allowing work to proceed safely and efficiently, and/or
3. During movement of vehicles and equipment in and around work zones (including access/egress), which may include off-road sites.

POLICY

A traffic incident management area is an area of a highway where temporary traffic controls are installed, as authorized by a public authority or the official having jurisdiction of the roadway, in response to a road user incident, natural disaster, hazardous material spill, or other unplanned incident.

It is a type of temporary traffic control zone and extends from the first warning device (such as a sign, light, or cone) to the last temporary traffic control device or to a point where vehicles return to the original lane alignment and are clear of the incident.

The primary functions of temporary traffic control at a traffic incident management area are to inform road users of the incident and to provide guidance information on the path to follow through the incident area.

Alerting road users and establishing a well-defined path to guide road users through the incident area will serve to protect the incident responders and those involved in working at the incident scene and will aid in moving road users expeditiously past or around the traffic incident, will reduce the likelihood of secondary traffic crashes, and will preclude unnecessary use of the surrounding local road system.

Examples include a stalled vehicle blocking a lane, a traffic crash blocking the travel way, a hazardous material spill along a highway, and natural disasters such as floods and severe storm damage.